

Career development

by MICHAEL DAVISON

If you don't build your dream, someone else will hire you to help them build theirs!

Dhirubhai Ambani

The first thing to realise is that, irrespective of the industry in which you work, the fundamental responsibility for your career lies with you. You are in charge. Of course you can let fate play its part, and without certain qualifications and experience you may find obstacles in your path. All of this considered though, it will be your determination and persistence that creates your career opportunities. It is important to think big about where you would like to be and to have the dream, but within this dream you need both a plan (which can change over time) and the stepping stones to achieve it.

In this chapter we explore the context in which careers in sport and exercise medicine (SEM), sports therapy and physiotherapy operate. This landscape has changed greatly in the past decade and is set to evolve even further in the next. The main driver has been the globalisation of medicine to support sport which has mirrored the globalisation of sport.

Furthermore we want to focus readers' attention, in particular that of future sports clinicians, on the key behaviours that are important to both potential employers and other people who will help with career progression.

We then reflect on some career advice provided by five members of the SEM community: two international sports physiotherapists; a young SEM doctor; one of the most controversial characters in SEM; and finally, a leading figure in injury prevention. All of their stories and advice provide invaluable real life guidance and inspiration.

DEVELOPMENT OF SPORT AND EXERCISE MEDICINE

Sport and exercise medicine has become an all-encompassing term for many subdisciplines and types of practitioners. Sports medicine is a multidisciplinary way of thinking based around three core themes or approaches:

1. Work as a team.
2. Communicate internationally.
3. Reach the top with your patients or athletes, where you seek to achieve the maximum functional recovery possible.

Across the world, SEM has developed at different rates and with different styles of leadership and personality. Now, however, the effect of globalisation is felt more strongly through the diaspora of the playing talent in sport and increased global travel to compete in both professional and amateur events, such as marathons and triathlons. Following this pattern, clinicians have begun to move internationally to practise and so cross-pollination is taking place. There has never been a better time for an international career in SEM.

The internationalisation of the discipline is not constrained to the travels of individual athletes. In fact, in the past decade international governing bodies such as FIFA (Fédération Internationale de Football Association) and the IOC (International Olympic Committee) have created international networks of clinics and scientific research centres or hubs (48 in the case of FIFA and nine in the case of the IOC), with the key driver of promoting international collaboration. Both groups also support large conferences, such as the annual Football Medicine Strategies Conference organised by Isokinetic Medical Group and the IOC World Conference on Prevention of Injury and Illness (held every three years). More than 1000 sports medicine practitioners from more than 70 countries come together at these conferences to share their experiences, network and seek collaboration for future multicentre studies.

This explosion in interest in the discipline has led to the need to establish a common method of communication,

and sports medicine has adopted English as its base language. To support and stimulate further cross-border sharing of information and experience, the *British Journal of Sports Medicine* (BJSM) has been proactive in co-opting well over 20 professional member bodies from countries as diverse as The Netherlands, South Africa, Australia and Switzerland, and has extended its editorial board to embrace 35 nations (at the time of publication).

Against this backdrop of movement of people and information, two other macro trends are positively impacting upon the cultural development of the discipline and the number of career opportunities. These are an increase in televised sport and its associated investment, and a growing understanding of the dangers of physical inactivity, which has led to the view that 'exercise is medicine'.

The adoption of exercise medicine in the sports medicine movement

In many countries, sports medicine has become known as 'sport and exercise medicine'. This term provides an apt description of the discipline, blending management of sports injuries and medicine in the sporting population with promotion of the beneficial effects of exercise and its positive contribution to world health.

Treatment of the effects of physical inactivity is outside the scope of this volume (see Volume 2: *Exercise Medicine*), but it is widely recognised that insufficient physical activity is a key risk factor for non-communicable diseases (NCDs) such as cardiovascular disease, cancer and diabetes.¹ There is mounting scientific evidence that physical activity may be a most important public health intervention to prevent NCDs. Evolving national health systems and structures, and public health policies, will increasingly shape the way in which investment is targeted to deal with the mounting burden of NCDs. This will lead to wider and more specialised career opportunities. The private sector will support this, especially where there are sporting injury comorbidities or a patient wishes to regain functional capacity. Additionally, as the population ages and the importance of sporting activities and participation among the older generation increases, the demand for specialised, private sports medicine services will increase.

Increased resources in sport

Television companies are now able to access global markets through digital and internet-based channels, resulting in a huge inflow of money into sport. This has triggered an increase in investment in both SEM and research. Not only are there now more resources available to club management to spend on player support and welfare, the economic rationale and business case for the role of SEM is better understood. Now, in a premier league football clinical team you would expect to see at least:

- a full-time SEM doctor
- two or three part-time doctors across all teams
- a ratio of one physiotherapist to five first team players
- osteopaths and chiropractors
- a sessional sports psychologist, a dietitian/nutritionist and a podiatrist
- sports therapists and other soft tissue specialists
- a medical administrator.

These roles, although they may be well paid, are also very demanding, often requiring availability across the full week. (See also Chapter 1.)

Proficiency in a second or third language

Another trend, in European football clubs in particular, is that language skills within the SEM team are increasingly required and valued. Considering that the playing group of most UEFA Champions League football teams is drawn from more than a dozen countries and mother tongues, it is difficult for SEM practitioners to communicate and build a bond of trust if they do not fully understand the cultural heritage of the players and their medical history. Added to this is the requirement to communicate effectively with the national team SEM staff for acute injury management, workload modifications and return-to-play (RTP) considerations. This additional workload on top of normal day-to-day club commitments is becoming more common in football as there are more international 'friendlies' and larger national team squads. Within football, Spanish and French are probably the most valuable second languages to learn. This allows for better communication with players from North and West Africa, and Central and South America.

Widening the scope of practice—dual qualifications and subspecialist courses

In addition to the increased demands that are placed upon SEM teams in professional sport, employers expect practitioners to have a wider vision than would be provided by a single educational experience. For example, within a rehabilitation-focused setting, classical physiotherapy training is not considered sufficient to help a patient to reach the maximum functional recovery possible.

If a practitioner has a secondary qualification, the scope of practice widens and there is a greater benefit for both the patient and the team. Among the staff at the Isokinetic Clinic in London practitioners hold the following dual qualifications: sports therapy BSc and fast-track physiotherapy BSc; physiotherapy BSc and sport and exercise medicine MSc; engineering science BSc and physiotherapy BSc; human movement science BSc and sports physiotherapy MSc. After the base university

education, the next development steps are in specialist courses and certification. For example, a SEM physician, doctor, or physiotherapist may be able to add value by doing an MSc in ultrasound or injections. This is a way of adding value to patients' treatment and at the same time becoming more employable. Another example might be a physiotherapist undertaking a course in the management of non-specific low back pain, if this is what is being seen in their clinical practice.

Sports therapy, sports and exercise science, and sports rehabilitation

There is a well-identified need for a blended skill-set and perspective, whether as an individual practitioner or among the wider SEM team. After graduating from undergraduate physiotherapy or physical therapies degree courses, the clinician has not yet received sufficient training to be effective in SEM settings. To meet this need, many clinicians undertake sports physiotherapy masters degrees.

There has also been an explosion in the number of new courses designed specifically to train students in the management and treatment of sports injuries. In the United Kingdom, sports therapy, sports and exercise science and sports rehabilitation courses have been among the fastest growing university courses over the past five years.

Overall, however, systems in public and private medicine and in team sports have not kept pace with this change, especially in the areas of state registration, professional indemnity insurance and identifying where they fit in the clinical governance model. In the United Kingdom, for example, because of a lack of state registration by the Health and Care Professions Council, (HCPC) sport and exercise clinicians are unable to be registered with private medical insurers (PMI) for work in the private sector and cannot always assume highly responsible roles in team settings. Fortunately it does seem that change is just around the corner and future accreditation processes will open up new avenues of career opportunities.

Men and women have a place in sport and exercise medicine

One final but important dimension to the development of SEM over the past decade has been the increasing participation of female doctors, physiotherapists, osteopaths and other clinicians in the discipline. Sport has historically been male-dominated and resistant to change. Women were (and many still are) disadvantaged in many settings. This is beginning to change, not simply in an age of equal opportunities, but as sporting teams recognise that medical teams need to be selected on merit and not prejudice.

KEY BEHAVIOURS FOR A SUCCESSFUL AND INTERESTING CAREER

My personal career path in sports medicine has been far from traditional. Three key points that contributed to my being in the position of responsibility I have are: (i) gaining skills that would *add value* to a sports medicine setting (business management in my case), (ii) adopting a *flexible attitude*—embracing change, and (iii) working to develop a network of top experts who support me and to whom I contribute. Turning the mirror to face the reader, I suggest the following five key behaviours are critical to developing a successful career:

- willingness to make yourself available to patients and colleagues
- open, honest and timely communication
- problem solving
- determination to reach the optimal result
- attention to detail.

Surrounding this is the development of self-awareness, which is built through an evolutionary process of maturation. These phases do not have to be sequential and they often run in parallel. There may be times when you reflect and revisit an earlier phase, especially as your self-awareness grows. Don't be afraid to go back to run forward.

In the first phase of your career you should seek to develop your technical skills and knowledge. This knowledge increases over time, but mainly as a result of practice. 'Miles on the clock' are important (Fig. 48.1).

Being an 'expert' is a dangerous time for a clinician. It may be a time of making clinical errors in advance of developing greater insight into one's limitations (Fig. 48.2).

A positive next step and second phase of your career is to develop your 'softer' skills, such as effective communication, negotiation, conflict resolution and problem solving (both through specific courses and also by talking with those close to you). To really develop in this area you

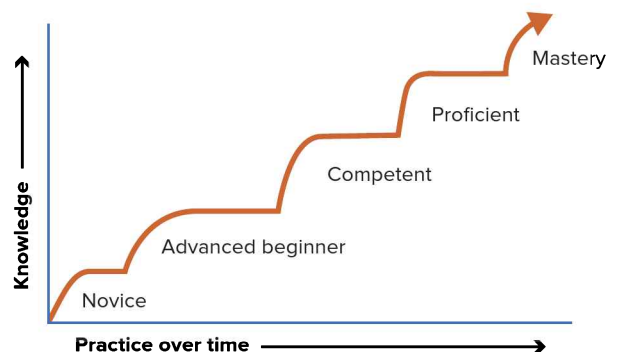


Figure 48.1 A schematic of how we can describe different levels of development as a health professional

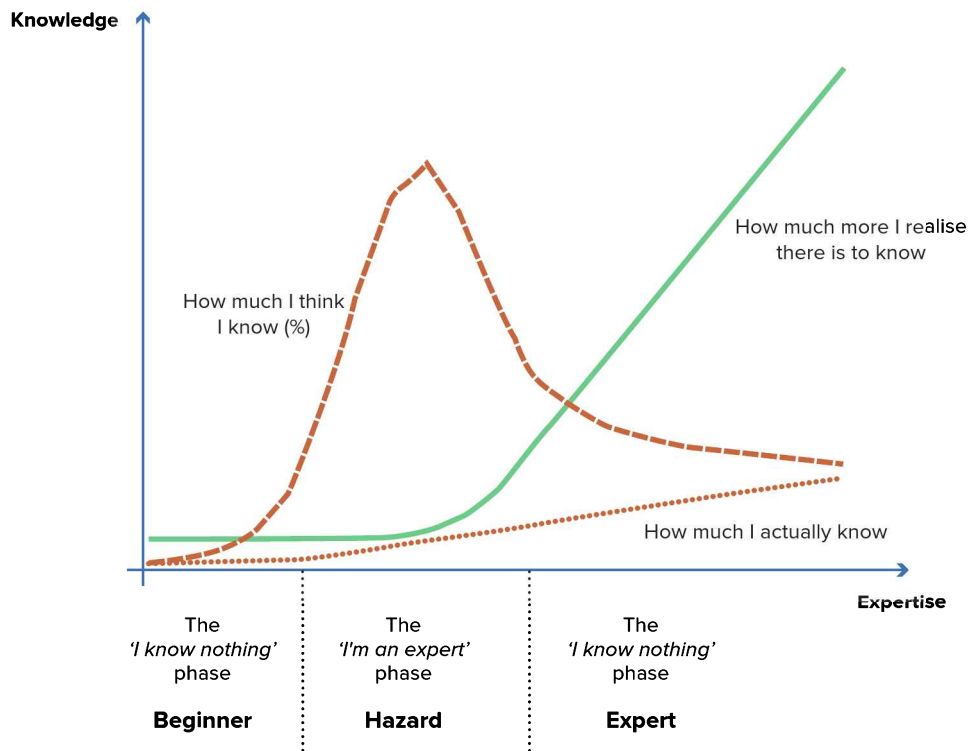


Figure 48.2 Depicts a common feeling where a learner's feeling of knowing nothing changes into feeling that he or she knows everything, before soon realising that even the most expert really knows very little

need not to be afraid of making mistakes and you need to seek out constructive criticism.

This then leads you into a critical SEM phase—that of understanding what it really means to work in a team. It means *collective action over self-interest*. Much easier said than done. There are countless examples of dysfunctional medical teams due to the inability of team members to find a simple working consensus.

At all times in your life and particularly as you start making your own decisions, you need to recognise the role of others close to you in your career development. This can be your parents, but is more likely to be your partner or spouse. Including that individual(s) in any decision making and being very clear about the likely impact on your time together (especially as sports medicine often involves travel with a team or long or anti-social hours in the working environment), will provide stability for you to grow. It is not a normal career that you are looking to develop, so you will need extraordinary support, understanding and encouragement around you.

Last, and while this sounds the easiest, the greatest challenge you will have is working on yourself. This is not in relation to developing knowledge or skills, nor is

it the teamwork that is required inside and outside work. It is about making sure you have a good relationship with yourself. One where you actively critique and score yourself, and drive yourself to live in the present, understanding the importance of the past and how it seeds the future. You need to have a relationship with yourself where you reflect on your actions honestly and with humility.

Complementing my perspective, here is an American perspective on how you could break into the world of sports medicine, especially team medicine, and succeed:²

- *Pay your dues.* Start working with local sports teams to gain experience.
- *Create visibility.* Make yourself known to team decision makers.
- *Communicate clearly.* Professional communication training is a plus.
- *Build relationships.* Sport is like life: it's all about relationships.
- *Create protocols.* A disorganised doctor is an ineffective doctor.
- *Establish a chain of command.* Let others know who to talk to and when.

- *Direct the flow of information.* A team needs to speak with one voice.
- *Clarify expectations.* Determine the needs and wants of each stakeholder.
- *Let the coaches coach.* Don't stick your nose where it doesn't belong.
- *View each player as an individual.* That goes for visiting players, too.
- *Be wary of the media.* If you must talk to them, stick to the facts.

LESSONS FROM AROUND THE WORLD

Below, five members of the international sport and exercise medicine community offer their advice to others about developing a career in sports medicine.

Ummukulthoum Bakare—football medicine enthusiast and sports injury prevention strategist West Africa

Ummukulthoum Bakare is based in Lagos, Nigeria. She is a member of the Medical and Scientific Commission of the Nigerian Olympic Committee. Alongside this role she is a senior physiotherapist at Lagos Teaching Hospital and one of the few accredited FIFA 11 For Health and FIFA 11+ (injury prevention program) instructors in Africa.

Like many successful SEM professionals before her, she understands that it is important to engage in continuous self-improvement and professional development.

Her advice to aspiring SEM clinicians is:

- *Practice makes perfect.* Be willing to shadow, undertake work experience, learn and do a large amount of volunteer work to get the necessary hands-on exposure and experience.
- *Never be tired of being a part of the knowledge continuum and always share information.* It does not make you any less intelligent!
- *Don't think you know everything and want to be in the spotlight.* Even professors are still learning—so should you. The light will shine on you when the time is right.
- *Procrastination is the thief of time.* Poor time management is the bane of any professional.

Dr Liam West—the rookie doctor Northern Europe and Australia

Liam is based in Melbourne, Australia, but completed his junior medical jobs in Oxford, England, and his undergraduate medical training in Cardiff, Wales. At the time of publication he was planning to start his specialist training in SEM medicine. He has a passion



Ummukulthoum Bakare

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Once you identify what goal you want to achieve as an upcoming sports physiotherapist, you have to seek out and surround yourself with senior colleagues who can help you on your path—mentors in the profession or a related field. An orthopaedic surgeon acted in this role early in my career (undergraduate university) and helped me identify my interest of specialisation in sport and exercise medicine. Let passion be your driving momentum and all the glitz and glamour may follow, but it is not guaranteed unless you make it count! Ummukulthoum Bakare

for increasing the opportunities and education in SEM for undergraduates and junior doctors. To help achieve this he founded societies: at local level while at Cardiff Medical School; at national level in the UK with USEMS (Undergraduate Sport & Exercise Medicine Society)—and at European level, with ECOSEP (European College of Sports



Dr Liam West

& Exercise Physicians). He is a Senior Associate Editor for the BJSM, produces insightful podcasts for the journal, and coordinated the BJSM 'Undergraduate Perspective on Sport and Exercise Medicine' blog series.

Liam recognises the value of developing an international network. During his undergraduate elective, he gained experience at the Olympic Park Sports Medicine Centre, Melbourne and the Stadium Orthopaedic and Sports Medicine Centre, Sydney, followed by time at the Aspetar Orthopaedic and Sports Medicine Hospital, Qatar.

Liam's suggestions for juniors wanting to develop a career in SEM include the following.³

- *You need energy and motivation, especially to seize opportunities.*
- *How do you get these opportunities?* By approaching medical or physiotherapy schools in the first instance

to see what they can offer. Ask if they have lectures on SEM and if they have faculty working there who are involved in sport.

- *When you are there, find out if there is a local student society.* If there is, then get involved. If not, make one. Aim to get on the committee to learn valuable skills while meeting key individuals in SEM.
- *Networking is key.* You can network in a general way (i.e. online) but the most effective way is at conferences—choose five names on the program and don't leave until you have their email addresses. Email them that night explaining it was nice to meet them and include something interesting about either what you said or did while with them. Then in the no-so-distant future, when you're looking for SEM experience, reply to that initial email and they will remember who you are from your story. This will help your request stand out from the weekly emails for shadowing opportunities the clinician receives. Wait until your chosen contacts are with at least two of their colleagues—you will meet more people this way for less effort. Networking should be fun!
- *Offer to work at conferences for free admission—pack bags, etc.* This gets you networking opportunities and plenary education for free!
- *Remember to network within your peer group, not just the big guns.* In 15 years time, hopefully you and your colleagues will be the leaders of the clinical and research arms of SEM so forge lasting relationships early.
- *Once networked, get your hands dirty and try to get as much experience as possible in sport.* You need to start viewing long weekends in the rain as good things. When shadowing, be polite, gently inquisitive, energetic and offer to help at every opportunity—do NOT be a burden and, importantly, do NOT be a fan. Doctors are employed to provide clinical care for athletes, not to scream abuse at a referee or celebrate a goal.
- *People don't owe you this experience.* You don't get it handed to you like cardiology or respiratory medicine during your undergraduate studies—you need to offer value to those giving the experience. Sitting in on a clinic may be achieved by doing an audit for the company, etc. Create your self-directed study modules or elective placements in SEM. Travelling for these experiences should not put you off.
- *Adding value can also be in the form of targeted learning.* If you are interested in a certain sport, learn about the common injuries—spines and fingers in cricket, groins and ankles in football, anterior cruciate ligament injuries in netball, etc.

Hans-Wilhelm Müller-Wohlfahrt—the team and celebrity doctor Bavaria, Central Europe

The oldest of our interviewees, Müller-Wohlfahrt (or MW as he is often referred to) is one of the most renowned sports doctors in the world. Aged in his 70s, he has been challenging the status quo and the orthopaedic medicine paradigm over the past 40 years. Often talked about with an air of controversy (due to his use of homoeopathic medicine to treat players), there is no doubt that he has pushed forward SEM's understanding of muscle injuries in particular. Along with his roles with Bayern Munich and the German national team, he has been a magnet for many celebrities from the world of sport and beyond. This includes Usain Bolt, Paula Radcliffe, Ronaldo, Steven Gerrard, Michael Owen and many more. Orthopaedically trained with a specialisation in sports medicine, his approach is one of conservative management and the avoidance of surgery until there is no other option.

For him, the advice for career development is simple and summed up in three principles shown in Figure 48.3. He believes that your career is defined by your ability to manage and win with patients.



Hans-Wilhelm Müller-Wohlfahrt

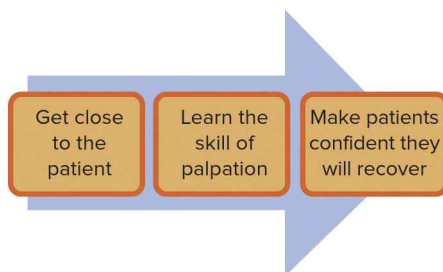


Figure 48.3 Dr Müller-Wohlfahrt's keys to career development

Rod Whiteley—sports physiotherapist who has moved across continents for his career Middle East via Australasia and Major League Baseball

Rod Whiteley is a sports physiotherapist working at Aspetar Orthopaedic and Sports Medicine Hospital in Qatar as a research and education physiotherapist. His primary clinical interest is in upper limb injuries, stemming in large part from having played baseball for over 30 years, from which point his PhD in throwing-related injury seemed almost inevitable. He was physiotherapist to the Australian Baseball Federation for 20 years prior to joining Aspetar and recently was the first non-American to be an invited keynote speaker at Major League Baseball's Winter Medical Meeting.

For him, the overall perspective to share is that 'there is no formula'. Reflecting on his career, he says the underlying theme 'is that you will need to very early suck up the idea that you are not going to be driving a Porsche, so if that's your motivation, get out while you can. If, on the other hand, your motivation is curiosity, an open ended career where things are constantly evolving, and the opportunity to be around other people who are also striving to get as far as they can largely through hard work, then come in, the water's fine'.

He also has some cautionary tales regarding fan syndrome, and losing sight of clinical and professional objectivity. He believes in the importance of honest and frequent self-appraisal and addressing one's own deficits. He is not sure if you could work properly in sport medicine if you are not athlete-centred rather than it all being about how wonderful you are as a clinician.

Finally he also recognises the phenomenon of 'How much I actually know'. He reflects: 'I keep reminding myself of whatever I was doing five years ago and how embarrassed I am about this now, which of course makes me realise that in five years from now I will think exactly the same about what I am now doing'.



Rod Whiteley

Roald Bahr—Professor of Sports Medicine The Nordic countries

Last but far from least, is one of the editors of this book, Roald Bahr, Professor of Sports Medicine in the



Roald Bahr

Department of Sports Medicine at the Norwegian School of Sport Sciences and the Chair of the Oslo Sports Trauma Research Center. He is also the Chief Medical Officer for Olympiatoppen and Chair of the Medical Department at the National Olympic Training Center. Additionally he is the Head of the Aspetar Sports Injury and Illness Prevention Programme.

Roald is more than these titles though. Professionally he is a world leader in sports injury prevention research having published more than 300 original research articles, review papers and book chapters, in addition to several books. More importantly though he is one of the most warm and generous human beings you could meet.

As is so often the case with those who have been very successful in their careers, the simple advice is built on a deep understanding of those things that really work. For him, the advice he would wish to share is shown in Figure 48.4.

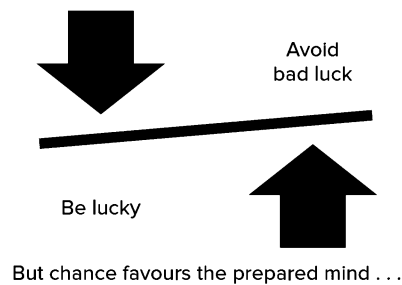


Figure 48.4 Professor Roald Bahr's keys to success

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References for this chapter can be found at www.mhhe.com/au/CSM5e